OPERATION REPORT

Name: [redacted] **Age : 52YR** **Sex : MALE**

IP No : ~~66943~~ 66216
Date & Time : 10-02-2026

Surgery Name : ROBOTIC CHOLECYSTECTOMY

Indication : FUC OF LIVER ABSCESS DRIANAGE,CHRONIC CALCULOUS CHOLECYSTITIS (CONTRACTED GALL BLADDER)

Surgeon : [redacted] **Assistants** [redacted]

Scrub Nurse : [redacted]

Anaesthetist : [redacted] **Anaesthesia : GA**

Incision : THREE 8 MM PORTS AND ONE 5 MM PORT

OPERATIVE FINDINGS:

SMALL BOWEL DENSLY ADHERENT TO ANTERIOR ABDOMINAL WALL
PIGTAIL CATHETERNOTED IN LEFT LOBE OF LIVER AND DENSLY ADHERENT TO OMETUM
GALL BLADDER CONTRACTED AND OCCUPIED BY LARGE STONE AND DENSE LY ADHERENT WITH OMENTUM AND DUODENUM PRESENT
CALOTS TRIANGLE- FROZEN AND DENSE ADHESIONS
C/S- GB WALL THICKNED LARGE STONE WITH MUCUS PRESENT

OPERATIVE STEPS-

-PNEUMOPERITONEUM ACHEIVED THROUGH VERRES NEEDLE AT PALMAR'S POINT
-PORTS PLACED
-ROBOTIC ARMS DOCKING DONE
-FINDINGS NOTED
-ADHESIONS SEPARATED
IN VIEW OF DENSE ADHESIONS AND FROZEN CALOTS TRIANGLE
GALL BLADDER OPENED STONE REMOVED AND STUMP CLOSED WITH 3-0 PDS
-GALL BLADDER MOBILIZED FROM LIVER BED
-HAEMOSTASIS SECURED
-24 CHEST DRAIN PLACED IN SUBHEPATIC REGION
- CLOSED WITH 2-0 NYLON
-

Post Operative Instructions:

NPO
IV FLUIDS- NS @100ml/hr
INJ SUPACEF 1.5 G IV BD
INJ.PAN 40mg IV OD
INJ. NEOMAL 3 AMP IV TID
GRBS-TID
SEND CBC,LFT C/M

[redacted]

[redacted]

CONSENT FORM FOR ANAESTHESIA

Name of Patient [redacted]	Age 52yrs
Sex : M / F	UHID 90634496
Diagnosis Cholelithiasis	Emergency/ Elective ✓
Surgery/ Procedure lap. Cholecystectomy	Anesthetist/s Name [redacted]

I, the undersigned, being over the age of eighteen (18) years and in proper state of mind do hereby authorize Dr..... [redacted] ..... and /or his/her team as may be designated by him/ her to provide recommended anaesthesia for the proposed surgery.

In case the patient is unable to give consent or is a minor, the same shall be given by the surrogate:

I,..... the undersigned being the surrogate decision maker of the patient ....., hereby authorize Dr..... and /or his/her team as may be designated by him/ her to provide recommended anaesthesia for the proposed surgery

Type of Anaesthesia, Expected Result, Technique and Risks including but not limited to:

Type of anaesthesia		
General anaesthesia	Expected Result	Total unconscious state, possible placement of tube into/vicinity of windpipe
	Technique	Drug injected into the bloodstream, breathed into the lungs, or by other routes.
	Risks including but not limited to	Mouth or throat pain, hoarseness, injury to mouth, lips or teeth, awareness under anaesthesia, injury to blood vessels, vomiting, aspiration of stomach contents, pneumonia, bronchospasm, difficult intubation, pulmonary hypertensive crisis, cardiac arrhythmias, cardiopulmonary arrest, hypotension, allergy to drugs
Spinal/Epidural Analgesia/Anaesthesia	Expected Result	Temporary loss of sensation over part of the body
With Sedation Without Sedation	Technique	Injected through a needle/catheter placed either directly into the fluid of the spinal canal or immediately outside the spinal canal.
	Risks (include but not limited to)	Headache, backache, buzzing in the ears, convulsions, infection, persistent weakness, numbness, residual pain, injury to blood vessels
Major/Minor Nerve Block	Expected Result Technique	Temporary loss of feeling and/or movement of a specific limb or area
With Sedation Without Sedation		Drug injected near nerves providing loss of sensation to the area of operation. Risks (include but not limited to) infection, convulsions, weakness, persistent numbness, residual pain requiring additional anaesthesia, injury to blood vessels, failed block
Intravenous Regional Anaesthesia	Expected Result	Temporary loss of feeli[redacted]nt of a limb

[redacted]

[redacted] CLN/V1-2021/14

With Sedation Without Sedation	Technique	Drug injected into veins of arm or leg while using a tourniquet
	Risks (include but not limited to)	Infection, convulsions, persistent numbness, residual pain, injury to blood vessels
Monitored Anaesthesia Care w/ Sedation	Expected Result	Reduced anxiety and pain, partial or total amnesia
	Technique	Drug injected into the bloodstream, breathed into lungs, or by other routes, producing a semiconscious state
	Risks (include but not limited to)	An unconscious state, depressed breathing, injury to blood vessel
Monitored Anaesthesia Care	Expected Result	Measurement of vital signs, availability of anaesthesia provider for further intervention
	Technique	None – No sedation is provided. Risks (include but not limited to) Increased awareness, anxiety and/or discomfort
General risks applicable to all types of anaesthesia	Risks include but not limited to	Remote possibility of infection, bleeding, drug reactions, and blood clots, loss of sensation, loss of limb function, paralysis, stroke, brain damage, heart attack and death.

Proposed Technique of Anaesthesia: J General Anesthesia

Alternative to the Proposed Anaesthesia: J G/A

I have been explained in great detail regarding the type of anaesthesia, reason for the selection, expected results, risks involved, in Hindi language. I have been given ample opportunity to clarify all my doubts and concerns. I consent to the use of such anaesthesia as may be deemed fit and necessary by the person responsible for these services, for the procedure that is to be undertaken.

I have been informed that a different anaesthesia may be considered necessary for any un-anticipated condition that may arise while being operated upon or during the immediate post-operative recovery and may require interventions to be performed on me or on the patient (in case of surrogate consent). I am also aware that the type of anaesthesia may be changed mid course of the surgery/ procedure as required.

I have been informed of the possibility of intra-operative or post-operative complications including ICU stay, estimated recovery time, anticipated activity level and possibility of additional procedures found during surgery/ anaesthesia or as a complication of the surgery/ anaesthesia.

I understand the importance of providing my health care providers with a complete medical history, including disclosure of any medications that I am taking, both prescription and over the counter.

I also understand that my use of herbal remedies, alcohol or any type of illegal drug may give rise to serious complications and must also be disclosed. I further understand that I should also disclose any complications that arose from past anesthetic exposure.

I consent to the anaesthesia services indicated and I authorize that it be administered by the ....., all of whom are credentialed to provide anaesthesia services at this health facility. I also consent to an alternative type of anaesthesia, if necessary, as deemed appropriate.

I acknowledge that I have read this form or had it read to me, that I understand the risks, alternatives and expected results of the anaesthesia service and I give my consent for the surgery to be performed under anaesthesia as deemed necessary.

Patient / surrogate decision maker	
Name [redacted]	If surrogate, state relationship to patient
Signature [redacted]	
	Witne[redacted]
Name [redacted]	Signa[redacted]
Mobile Number [redacted]	
	Anaes[redacted]
Name [redacted]	Signa[redacted]
Date and Time 10/2/26, 12pm	

[redacted]

INFORMED CONSENT FOR SURGERY / PROCEDURE

Patient name:	Sex: Female Male
Diagnosis Cholelithiasis Emergency/elective:	Age 52yrs
Consultant	UHID: 90634496

I, the undersigned being over the age of eighteen (18) years and in proper state of mind do hereby authorize Dr. _____ and /or such associates and assistants as may be designated by him to perform the advised surgery under recommended anesthesia

In case the patient is unable to give consent, the same shall be given by the surrogate:

I, _____ the undersigned being the surrogate decision maker of the patient _____, hereby authorize Dr. _____ and /or such associates and assistants as may be designated by him to perform the advised surgery under recommended anesthesia

Name of surgery/ procedure(s):
lap. Cholecystectomy

Reason(s) for the selected treatment modality is expected to include (but not limited to) the following benefits:

a. Improvement of symptoms
b. Better outcome
c. To save life
d. Improve quality of life
e. Others _____

Alternative treatment modality: _____

My physician(s) has fully explained to me the condition requiring treatment and the nature, purpose, risk and benefits of the operation(s) / procedure(s), possible alternative methods of treatment, including non-treatment, and the possibility of complications. I was given the opportunity to ask questions and any such questions were answered to my satisfaction, in Hindi language. No guarantee or assurance has been given by anyone as to the results that may be

obtained. I am aware that the practice of medicine and surgery is not an exact science. I am aware that the outcome varies from one individual to another.

Risks: I do understand that any operation/procedure involves some risks and hazards like infection, bleeding, allergic reactions etc. Some of them may be serious and possibly fatal. My doctor has explained specific risks involved in this procedure to me which may include one or more of the following:

Common risks and complications include but not limited to:

a. Bleeding from the wound or from the operated site within, which may need blood transfusion and /or a re-exploration if bleeding persists/recurs
b. Infection
c. Injury to surrounding organs
d. Nerve injury
e. Allergic reaction
f. Wound may not heal normally. Occasional secondary infection could set in, especially with inadequate blood sugar control (1-10%)
g. Blood clot in the leg (DVT), rarely part of which may break off and move to the lungs.
h. Fatality during surgery, or post surgery
i. Some complications may require a redo surgery for diagnostic or therapeutic purposes
j. Long ICU or hospital stay due to any complication that may or may not be foreseen
k. Others_____

This procedure could carry a higher risk under the following conditions:

a. Age
b. Associated heart diseases
c. Untreated/uncontrolled diabetes /hypertension
d. Chronic lung/kidney disease
e. Peripheral vascular disease/ carotid disease
f. Procedure performed on emergency basis
g. Others_____
_____

Refusal to undergo the procedure/surgery may result in

a. No relief of symptoms
b. Recurrence/increase of the problem
c. Fatality
d. Others--_____

If implant is used

I have been explained about the___implant that is to be used for my surgery. I have been explained that in some instancesthere is a chance of infection and this may necessitate a prolonged ICU stay, removal of the implant and redo surgery at a later date. I havebeen given information on the advantages and disadvantages of various implant options and I have clarified all my doubts with my doctor

[redacted]

PRE ANAESTHESIA CHART

Department of Anaesthesiology

IP No : VZ1IP26/66216 UHID : 90634496

[redacted]

Sec. Doctor:

Proposed Surgery: lap. cholecystectomy.

Preoperative Diagnosis: Liver abscess - s/p pigtail catheter ; Calculus cholecystitis

B.P.	H.R.	R.R.	Temp.	Height	Weight	RISK STRATIFICATION
130/60	116/min	14/min	N	176cm	80kg	ASA 1 (2) 3 4 5

Laboratory Data:

Hb 9.7	RBS/FBS 84	Protein	HIV		X ray NAD Others
PCV	Urea	Alb 3.4	HbsAg NR		ECG NSR
WBC 9390	Creat 0.5	Total Bil 0.6	HCV		Echo EF-68% NO RWMA
Plts 3.52	Na	sGOT 29			
PT 13.4	K	sGPT 17	Bl.Group/Typing A +ve		Stress/Angio
APTT 33.4	Cl	ALP 200	Thyroid		
INR 1.21	Ca	Amylase	Profile		

Allergies: —

CVS/ Physical activity: HTN° / CAD° Effort tolerance ~4METS

Resp: URTI° / Br. asthma° / TB° Tobacco chewer Smoker

Diabetes: + ;

Renal: UAP good

CNS:

Hepatic/GE: Jaundice +

APD +/-

Others: -

Vysya/Non-vysya

Past anaesthetic history: —

Physical exam: A° / I° / Pe°

Airway: MP 1 (2) 3 4 Mouth Opening: Adequate Mentohyoid distance: 6cm Neck: normal movements Teeth: N

Lungs: NVBS BAE + VC - OK SpO2-97% on RA

Heart: S1S2 + no murmurs

CNS: pl. c HMF (N)

Pupils: EVM:

Others: Pallor: +/- Venous Access site: Peripheral Spine Exam for Regional: N

ANAES. PLAN: MAC/REGIONAL/GA-ETT/LMA	PROPOSED POST-OP PAIN RELIEF:	Peri-op Plan explained to patient Y/N
	Multimodal analgesia	

Current Medications:	Informed Consent: Standard / High Risk		→ Reserve 10 PRBC 20 FFP
1. OHA	NBM from: 6 hrs before Sx	Blood/blood products:	
2.	DVT Prophylaxis:	Medications:	→ elt, tyR.
3.	Pre-op Consultations:		
4.			T. pantop 40mg OD 6AM on the day of Sx
5.	PAC done by		T. perinorm 10mg
6.	Dr.		
7.	Designation:		

IV fluids - NS/RL 100ml/h from 6AM on the day of Sx

— P.T.O.

[redacted]

[redacted]

MRD/CLN/V1-2021/78

IMMEDIATE PRE-OPERATIVE EVALUATION			Anaesthesia plan modified: Yes /No
Pulse	B.P.	SPO_2	If Yes, reason:.....
90/min	150/80	98% on RA	.....
Date: 10/2/26			Signature: [redacted]
Time: 12pm			Name:.....

REVIEW PREANAESTHETIC EVALUATION NOTES

→ FBS on the day of Sx
withhold OHA on the day of Sx
→ Incentive spirometry

10/2/26
Case reviewed
Can be taken up for Sx

Signature: [redacted]
Name:
Date: 10/2/26
Time: 12pm

DATE 10/2/26

PRE-OP DIAGNOSIS cholelithiasis SURGERY lap. cholecystectomy

SURGEON [redacted] ANAESTHESIOLOGIST [redacted]

N_2O/AIR/O_2	1.4/0.6 L/min			
ISO/SEVO/DES	1%			
DRUGS: Midazolam 1mg IV, Glyco 0.2mg IV, Pethidine 1mg IV, Propofol 100mg IV, Vecuronium 7mg IV				
FiO_2/SpO_2	100%	100%	100%	100%
Et CO_2	30	30	30	30
EKG	SR	SR	SR	SR
CVP/WEDGE				
URINE				
EBL				
FLUIDS/BLOOD BL PRODUCTS	NS 1L, PRL 1L			
	2:20PM	2:50PM	3:20PM	4PM

Notes

Antibiotic Meropenem 1gm IV

Time Given : 2PM

Inj Tranexa 2cc IV

Inj Zofer 4mg IV

Inj Neomol 900mg IV

Inj Tranexa 1gm IV

100ml

Anaesthesia — Start — Finish

I Intubation

P PREP

O OP START

O OP END

Ex Extubation

BP: V Systolic, Λ Diastolic, X mean

heart rate

Tourniquet up T1

Tourniquet down t

REST: O Spont

AR Assisted

CR Controlled

RATE

TV

PIP

PEEP

TEMP CENT (T)

☑ EQUIPMENT CHECKED AND FUNCTIONAL
☑ BP
☑ CUFF SITE (R) arm
☐ ART SITE
☑ ECG LEAD
☐ TEMP SITE
☑ FiO_2 MONITOR
☑ AGENT MONITOR
☑ CAPNOGRAPH
☑ VENTILATOR
☐ NERVE STIMULATOR
☑ OXYGEN SATURATION

LAB VALUES
COMMENTS/SYMBOL
TIME
PH
$PaCO_2$
$PAO2/FO_2$
HCO3/BE
NA/K

TIMES
ANAES START 2:20pm
OP START 2:30pm
OP END 5:10pm
END ANAES 5:20pm
LEAVE OR 5:30pm

INDUCTION
☑ IV ☐ INHAL
☐ IM ☐ OTHER
☐ $PREO_2$ ☐ CRICOID PRESSURE

MASK (LMA) Size
AIRWAY ☐ ORAL ☐ NASAL
ETT
☑ ORAL ☐ NASAL ☑ CUFF
☐ TRACHEOSTOMY
☐ TOPICAL ☐ DRUG
%..... ml
☐ TRANSTRACHEAL
DRUG..... %..... ml
☐ AWAKE ☐ RAPID SEQUENCE
☑ DIRECT VISION ☐ BLIND
☐ FIBEROPTIC ☐ STYLETTE
BLADES..... ATTEMPTS.....
DIFFICULT WHY?

REGIONAL
☐ EXTREMITY
SPECIFY
☐ SPINAL
☐ EPIDURAL ☐ CAUDAL
☐ PUMP
☐ OTHER
POSITION
SITE
NEEDLE SIZE..... DEPTH.....
PARAESTHESIA ☐ YES ☐ NO
SPECIFY
CATHETER AT SKIN..... (CM)
DRUG/DOSE..... ☐
TEST DOSE
ANAES LEVEL
COMMENTS

POSITION
☑ PRESSURE POINT CKD
EYE CARE
☐ OINT
☑ TAPE
☐ PADDING

TEMP CONTROL
☐ HUMIDIFIER
☐ BED WARMER
☐ LIGHTS
☐ HEATERS
☑ HUGGERS
☐ BLANKET
☐ MATTRESS
☐ OTHER

☑ GENERAL
☐ MAC NO DRUG
☐ MAC WITH DRUG
☐ REGIONAL
☐ LOC BY SURG

LINE (SIZE AND LOCATION)
☐ CVP
☐ PA
☐ ART
☑ IV 18G (2) UC
☐ IV
☐ IV

☐ BILAT=BS
☐ SEMICLOSED CIRCLE
☑ CLOSED CIRCLE
☐ NON REBREATH

TRANSP[redacted]
RELAXA[redacted]
☐ TRAIN[redacted]

ADVERSE ANAESTHESIA EVENT : YES / NO

IF YES; SPECIFY :

Sign[redacted]

Nam[redacted]

Anaesthesia: General ✓ Epidural Spinal Regional Block

Anaesthesiologist: [redacted] Surgeon: [redacted] Procedure: Lap. cholecystectomy

Received in PACU by: [redacted] Time in: Time out:

POST ANAESTHESIA RECOVERY SCORE	IN	MINUTES 30	60	90	OUT	SCORING INTERPRETATION
Able to move 4 extrermities voluntarily or on command = 2 Able to move 2 extremites voluntarily or on command = 1 ACTIVITY Able to move 0 extremities voluntarily or on command = 0		0	1	2		A MINIMUM TOTAL SCORE OF 8 IS REQUIRED FOR DISCHARGE (EXCEPTIONS TO THIS ARE TO BE EXPLAINED IN THE SPACE BELOW BY THE DISCHARGING PHYSICIAN)
Able to deep breath and cough freely = 2 Dyspnea or limited = 1 RESPIRATION Unable to deep breath and cough freely = 0		0	1	2		
BP +20 of Pre anesthetic level = 2 BP+20-50 of Pre anesthetic level = 1 CIRCULATION BP+50 of Pre anesthetic level = 0		0	1	2		
Fully awake = 2 Arousable on calling = 1 CONSCIOUSNESS Not responding = 0		0	1	2		
Pink = 2 Pale, dusky, botchy, jaundiced = 1 COLOUR Cyanotic = 0		0	1	2		
TOTAL		0	5	10		

DATE	TIME	Medications (Drug dosage, route)	POST OPERATIVE INSTRUCTIONS
			1. Analgesia Inj Neomol 900mg IV
			2. Analgesia Buprenorphine patch
			3. Fluids NS/RL 100 ml/hr
			4. Anti emetics
			5. PCA/Epidural/ I.V. Infusion
			6. Other: NBM till further orders
			[redacted]

Received on unit by (Name & Signature): (Nurse): [redacted]

Transferred to unit by (Name & Signature): [redacted]

Evaluated and discharged by (Name & Signature): Dr.

Registration Number of evaluating Doctor:

Discharged by (Name & Signature): (Nurse):

[redacted]

Date :- 10/2/26

Procedure :- Laproscopic Cholecystectomy

[redacted] T/A :- ↓G/A [redacted]

SURGICAL SAFETY CHECK

Patients Name: [redacted] I.P. No. 66216/90634496 Age/Sex: 52yrs/m

Before Induction of anaesthesia
(With at least nurse and anesthetist)

Has the patient confirmed his/her identity, site, procedure, and consent?
- ☑ Yes

Is the site marked?
- ☑ Yes
- ☐ Not applicable

Is the anaesthesia machine and medication check complete?
- ☑ Yes

Is the pulse oximeter on the patient and functioning?
- ☑ Yes

Does the patient have a:
Known allergy?
- ☑ No
- ☐ Yes

Difficult airway or aspiration risk?
- ☑ No
- ☐ Yes, and equipment/assistance available

Risk of>500 ml blood loss (7ml/kg in children)?
- ☐ No
- ☑ Yes, and two IVs/central access and fluids

[redacted]

Before skin incision
(With nurse, anaesthetist and surgeon)

- ☑ **Confirm all team members have introduced themselves by name and role.**
- ☑ **Confirm the patient's name, procedure, and where the incision will be made.**

Has antibiotic prophylaxis been given within the last 60 minutes?
- ☑ Yes
- ☑ Time of Skin incision 2:30pm
- ☑ Time of antibiotic administered 2pm
- ☑ Name of the antibiotic Inj. Monocef 1gm
- ☐ Not applicable

Anticipated Critical Events
To Surgeon:
- ☑ What are the critical or non-routine steps? N/A
- ☑ How long will the case take? 2-3hr
- ☑ What is the anticipated blood loss? N/A

To Anaesthetist:
- ☑ Are there any patient-specific concerns? N/A

To Nursing Team:

[redacted]

Before patient leaves operating room
(With nurse, anaesthetist and surgeon)

Nurse Verbally Confirms:
- ☑ The name of the procedure Laproscopic Cholecystectomy
- ☑ Completion of instrument, sponge and needle counts yes
- ☑ Specimen labelling (read specimen labels aloud, including patient name) yes
- ☐ Whether there are any equipment problems to be addressed

To Surgeon, Anaesthetist and Nurse:
- ☑ What are the key concerns for recovery and management of this patient? NIL

[redacted]

Anesthetist (Sign & Name) [redacted]